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Henri Cartier-Bresson, Children in Seville, Spain, 1933



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The Editorial Note

This issue starts from a fairly obvious question that somehow still gets missed: what if disability isn't just about individuals, but about the way the world is organised around them?

Across these essays, disability shows up in ways that are often brushed aside as inconvenience, as delay, as something to be quietly “managed.” There are stories of allergies that are treated as overreactions, of inclusion that exists more in policy than in practice, and of environments that expect people to adjust endlessly, while calling it “accommodation.”

What these pieces do, in different ways, is refuse that surface-level comfort. They point to something more structural: that ideas of normalcy, productivity, and independence aren't neutral, instead they decide who fits, and who is expected to adapt.

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THE WORLD THAT DISABLED US

PALASH AKSHAY JAIN

Every essay begins with a small act of persuasion. The writer first attempts to capture the reader's attention, and then precariously arranges words into arguments in the hope that, by the end, the reader will see the world slightly differently. I thought of beginning this one as a fictional tale. It only seemed fitting. After all, the real world is built on ideas far more imaginative than fairy tales.

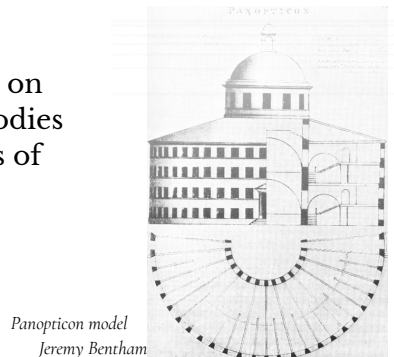
Disability is one such fiction.

With the rise of industrial capitalism came the invention of the “able-bodied worker,” as human bodies began to be measured by their productivity. Those bodies that lay outside the norms of speed, efficiency and profit maximization, came to be categorized as “disabled,” which over time emerged as a structural category.

Capitalism, in turn, rests upon another powerful fiction in liberalism. It was John Locke that brought forth the universality of human nature as “rational, independent, and self sufficient;” and those who were irrational, dependent, or incapable of self-governance were denied full humanity.

As Stacy Clifford notes, Locke even drew upon the category of intellectual disability, what he called “idiocy”, to define rationality by its absence. And therefore, scholars of disability studies have written about Locke being the reason for the medicalization of intellectual disability.

According to Foucault, social order depends on the creation of ‘docile bodies,’ which were bodies trained to meet the normative requirements of their environments and conforms to the expectations placed upon it.



*Panopticon model
Jeremy Bentham*

Though the theory is most often used by feminists to describe the surveillance of women's bodies, it helps explain how individuals came to be described in terms of their deviations from the ideal body and thus seen for they are not, as "disabled", and not for what they actually are.

In *A Very Capitalist Condition*, Roddy Slorach poses the argument that the very idea of "disability" is a relatively recent invention, and that in pre-political societies such a distinction never existed. He credits the rise of disability to the rise of the class society, especially in the era ushered in by the industrial revolution.

Marx too wrote of disability. He described the "*reserve army of labour*" which is a population of unemployed and underemployed workers whose very existence disciplines those still employed. Among this surplus population were those unable to work due to impairment, whom Marx described as the "lowest sediment" of the working class, pushed into pauperism. He wrote of how their suffering was intentionally caused, for the visible impoverishment of disabled people served as a warning to conform or risk joining them.

Disability, then, is not merely a medical condition or a personal tragedy. It is a socially produced, fictional construct, that is upheld by economic structures, medicalisation, and through institutions of power that place profit over people. And yet, while disability is produced collectively, it is experienced individually and endured as if it were a personal misfortune rather than an intentional structural outcome.

This is why disability is not simply a failure of compassion or awareness. It is a failure of structure. Not one that can be resolved through shifts in attitude, but one that demands economic reconstruction.

People often act like such a world is too fictional and impossible, because "humans won't cooperate," but perhaps one should open their eyes to the amount of cooperation it takes just to make a pair of socks. We already collaborate on a massive scale; the issue is simply who benefits. The fiction, therefore, is not that humans can cooperate, but rather that capitalism is "individualistic."

Disabled individuals remind us that **human life is not built on independence, but on interdependence.**

Yes, “disability” is a fictional invention, and so are the systems that produce it. Ideas do not need to be true to shape reality. They only need to be believed.



Metropolis (1927)
Fritz Lang

ASTHMA AND ALLERGIES

PJT SHAH

This essay adopts an extremely personal tone in reflecting on daily life with asthma and severe food-allergies, the socialisation process and the interaction of privilege with disability. It adopts a comparative legal aspect and refers to Sen's capability approach in arguing for the recognition of asthma and allergies as disabilities in India. After reflecting on the shortfalls in Indian Law, it proposes to widen its purview in a manner appropriate to the Indian context.

When I was given semi-solids as a 4-month old baby, that was when I had my first incident of food allergies. I reacted to pulses such that I had rashes and difficulty breathing. A few months later, my parents heard my first wheeze as I reacted to the dust and red soil being carried by the wind in Matheran - a hill-station. As an infant, my parents knew that consuming any allergen would cause itching on my tongue, lips and skin rashes. However, little did they know that far worse was yet to come.

There have been enough instances where the wider community has added to my problems and lack of awareness and apathy has led to social exclusion. At social gatherings, people would touch me or pull my cheeks despite my parents' refusal. That would culminate into skin rashes that could get quite nasty depending on the severity of the allergen. Perhaps people did not understand the meaning of 'touch-sensitivity' until they had seen it for themselves.

However, a toddler that could not speak up for himself would be the one paying the price for others' lack of *basic understanding* - let alone empathy.

Then, came the tip of the iceberg. Once in my ancestral town in Kutch, Gujarat, we were visiting someone else and my mom had informed them of my allergies. Despite her clear instructions to not give me any food (without her prior permission), the hostess still decided to give me some 'sev' just to 'play with'. Not knowing what it was, I had some.

I struggled to breath. At first, my mother thought that I was simply being playful or seeking her attention but as she looked at me, she saw that my face had flared up. That is when she lost her cool and the mother in her could take it no more. She confronted the hostess and left in a fit. The time till the reaction settled was indeed very difficult for her as she panicked about finding decent medical facilities in a rural area at a time when even the most basic hospital was two hours away. That was the day my mother decided not to take me to social gatherings or other people's houses anymore. People did not explicitly exclude us but it was their behaviour, attitude and such repeat experiences that distanced us.

Fast forward a few years, I did my composite allergen panel blood test and became aware of even more allergens. It was moderately expensive and in hindsight I realise how privileged I was to have undergone the test. It is unfortunate that diagnosis of allergens is also a function of privilege. While a cheaper option exists, it takes a toll on the person concerned. This involves trying out different potential allergens and maintaining a food journal to identify the specific allergens that affect the person concerned. However, even food journaling as a method of allergen identification is in its nascent stage. Earlier, people would go through a brutal process: a technician would prick the person one-by-one with each potential allergen from a comprehensive panel; then, the reaction would be observed to identify the specific allergens.

This process was extremely dehumanising whereby the person would be reduced to a mere test subject _____

and have to undergo a traumatic experience. At times, such procedures would lead to life-threatening allergic reactions that would necessitate the administration of epinephrine and being placed under observation at an ICU. Hence, it is a matter of immense privilege to have been able to afford to simply identify my allergens while bearing minimal suffering.

For years, I had a difficult time dining-out due to the lack of allergy awareness and sensitivity at food joints in Mumbai and across most of India. There have been enough incidents wherein the staff even at a five-star hotel or a fine-dine was unable to cater adequately and caused a mishap. Fortunately, this has changed over the past few years as there has been increasing awareness and sensitivity.

With the spread of dietary choices like vegan and keto to India, there has been growing awareness surrounding allergies. However, this awareness remains restricted to certain cities and the ground reality has not changed much in rural India. Even in cities like Mumbai that have begun to accommodate food allergies, such consideration is mostly restricted to places having a higher price-point.

Hence, eating out with food allergies is also a function of privilege.

Such a lack of affordable options, restricts the mobility of people with food allergies.

Moreover, it impacts and alters the socialisation process often resulting in exclusion unless they are able to find a circle of privileged people that can afford to regularly eat out at expensive places.

More importantly, the restricted mobility can have broader and more significant implications for higher education and career trajectories. Students that relocate for higher education - domestically or abroad - often depend on outside food due to time constraints. A similar pattern can be seen in people that migrate for careers. However, severe food allergies - more specifically, the risk of cross-contamination cannot be accommodated by most facilities that produce food on a mass-scale. Moreover, eateries that are able to cater to such concerns tend to be priced such that they may not be a viable option in the long-term for a daily basis. Once again, there is a strong case for privilege mitigating the impact of disability; by being able to spend on safe outside food instead of cooking themselves, people are able to perform better at work or studies. Clearly, the incidence of food allergies (as a disability) is borne by those affected: be it in terms of higher prices for safer food (if they could afford it) or the time that they would have to devote to cooking instead of studying or being at work. There are limited measures whatsoever to redistribute the incidence/ lessen the burden of food allergies in this regard.

Personally, I could not consider applying to colleges in India outside my home city due to the lack of any institutional frameworks to address the challenges faced by me.

In India, there is a lack of awareness around food allergies.

*The lack of formal **acknowledgement**
has led to a lack of
specific targets and policies.*

In August 2025, the Department of Empowerment of Persons with Disabilities (DEPwD) rejected the inclusion of asthma among eight other conditions in the list of Benchmark Disabilities recognised under the Rights of Persons with Disabilities (RPwD) Act. It is known that asthma and allergies tend to occur together. Hence,

had this resolution gone through, it could have been a turning point for people like me that suffer from both asthma and food allergies.

From a comparative legal aspect, the demand for recognition of asthma and food allergies as disabilities is not an unreasonable one. The Equality Act 2010 of the United Kingdom and the Americans with Disabilities Act (ADA) of 1990 of the United States both recognise both these conditions as disabilities.

While, Indian Law is quite similar to English Law, there is a departure in matters of jurisprudence as far as legislation pertaining to disability is concerned. UK Laws have laid down certain criteria that qualify a certain condition as a disability whereas Indian Laws have simply prescribed a benchmark list of disabilities with minimal support for those falling outside its purview. Both asthma and food allergies have a “substantial, long-term adverse effect on a person’s ability to carry out normal day-to-day activities” hence, are recognised as disabilities under the Equality Act 2010. However, the Indian panel (DEPwD) noted that asthma is a chronic but treatable disease and did not consider it as resulting in long-term disability. It is appalling that the two countries differ significantly in their opinions on such conditions despite the fact that Indian Law and jurisprudence are shaped significantly by English Law and Common Law Principles. The value of human life or the weight placed on such conditions ought not to vary depending on whether the country in question is developed or developing. Perhaps, this differential treatment could be attributed to the extent of availability of resources and provisions for spending on social welfare.

Evaluating the issue using Sen's capability approach, asthma and allergies impact both the capabilities and functionings of an individual by reducing their mobility through the way in which they interact with society, institutions and even with their identity as they navigate their daily lives. Hence, these conditions pose a developmental challenge by reducing a person's capabilities as well as the choices available to them thereby diminishing their overall personal freedom. Having classified these conditions as issues pertinent to economic development as per the capability approach, it could be argued that it would be reasonable to classify these very conditions as disabilities at an individual-level. Moreover, this resonates with the United Nations' definition of disability, "long-term ... impairments that, in interaction with various barriers, hinder their full and effective participation in society on an equal basis with others...".

*The UN definition moves away from focussing solely on medical conditions
to
recognising attitudinal and environmental barriers.*

This lays down a more holistic criteria for the definition of disability: participation in society that is full, effective and on an equal basis. This definition is enshrined in the UN Convention on the Rights of Persons with Disabilities (UNCRPD), which India ratified in 2007. Furthermore, the preamble to RPwD 2016 asserts that its purpose is to give effect to the UNCRPD. However, the previous comparative legal analysis shows that the views and interpretations of the Indian panel that are in contravention to the UNCRPD result in an enforcement that is only in letter!

If India wants to enforce the UNCRPD in true spirit, it must recognise these conditions as disabilities. However, this may not be feasible under the prevailing socio-legal infrastructure. Any proposal that seeks to promote economic development for all must be well-informed by the local context, pre-existing conditions and the grassroot reality.

I endeavour to do exactly that in pitching a solution that checks this criteria and is compatible with the Indian institutional framework. I propose broadening the purview of RPwD 2016 without causing any significant increase in the costs that it imposes. This could be achieved by establishing a new nomenclature for disability with the intensity of deprivation (capabilities affected) being the distinguishing criteria.

Benchmark Disabilities as defined under the current regime could be retained as the category of disabilities leading to the highest deprivation. More disabilities could be identified and classified in categories such as 'medium deprivation', 'low deprivation'. The range of benefits provided to people with disabilities could be mapped onto the level of deprivation inflicted upon them by their disability. Doing so would curb the costs of implementation while broadening the scope. Given the mitigating effect of privilege on disabilities, additional provisions of excluding the creamy layer from certain benefits could be considered to further curb costs and prevent wrongful enrichment.

For example, under the current regime benefits provided to persons with disabilities include but are not limited to reservation in government jobs and premier educational institutions across the country; I believe that my individual circumstances do not warrant me such special considerations. However, flexibility in attendance at educational institutions as already envisaged under RPwD is a benefit that could be extended to someone like me. I am aware of someone who is asthmatic and was low on attendance as they had to stay at home to nebulise frequently. However, the college insisted that they rather nebulise in college so as to attend regularly. This demonstrates complete apathy. *We must introspect what passes off as inclusion and whether it is truly enough?*

Furthermore, this person would cough frequently during their classes and also vomit at times thereby causing disruption and being a nuisance to the students at large. Such incidents lead to negative societal attitudes towards persons with disabilities. This incident also highlights the need for emotional sensitivity in leadership: be it educational administrators or managerial staff at a workplace.

Being a second year undergraduate student, I could not develop policy notes on this issue. However, I endeavour to continue working on it in whatever capacity I can. I solemnly believe that policy advocacy cannot be successfully attained without societal awareness and community action.

Having studied at Aditya Birla World Academy, I have witnessed the impact that Dr. Neerja Birla has had on mental health advocacy. She has been a beacon of inspiration that has shaped my perception of self as well as my functionings and capabilities.

Growing up, I aspire to become for asthma and food allergies in India what Dr Birla has become for mental health advocacy. I look forward to creating grassroots level action and advocating for people with asthma and food allergies. Perhaps, I may write a policy note someday; but what truly matters is achieving real-world impact that uplifts, that improves the condition of people, and

that contributes to the *wider* purpose of human
development.

BEYOND BEING INCLUDED

NIDHI DAHANUKAR

If you were someone who got taken to the optometrist during school years or if you knew someone who did because they could not see the blackboard clearly, the problem was solved by getting a prescribed set of glasses. There were no comments about trying harder to see or praises about seeing clearly again. It is not assumed that needing spectacles would make the wearer smarter or less intelligent. Glasses are mere assistive devices that allowed life to continue without commentary while correcting the mismatch between the body and the environment.

Yet when the assistive device is a wheelchair, a hearing aid or simply extra time in an examination hall, the emotions attached to it shift. The support or assistance is then categorised as “**Special treatment**”. Accommodation is then termed as generosity and the independence is then questioned. The act of adjustment is not where the difference lies but the hierarchy that is inherently attached to the certain kinds of difference.

If needing assistance were an interpreter of inferiority, everyone who wears glasses would be looked down upon. But they are not. Yet only when the assistance required is visible or structural, it quietly reshapes how competence is judged. *The hierarchy does not lie in the device, it lies in our interpretation.*

Accommodation to differences is not a problem that the human race faces anymore. Societies are capable of accommodations. *Rather, accepting disability without attaching inferiority is where the real problem lies.*

The inclusion of people who are different than us in our daily lives has now become a routine. Prayer rooms in offices, availability of food items based on different preferences, adjustable podiums, chairs for people with all heights are some of the examples. Although they are quite simple, we do not term these accommodations as burdens. If anything, they are the logistical decisions that help diverse people to participate without being evaluated based on the competence or worth of the participants. In these cases, support is seen as design and not charity.

When disability is introduced in such places, it exposes the myth of neutrality. The person with disabilities is expected to justify their participation and be patient while the approvals are being processed. Extra time in exams show that speed has an artificial privilege. Wheelchairs show that stairs were not all-inclusive or universal. Inclusion policies try to retrofit the support on the very systems that were not intended for diversity. The problem here is not the support that is provided; it is the implication that providing support lowers the standing.

Contemporary economies intensify this stratification. Productivity is equated with morality, value becomes synonymous with efficiency and independence is glorified. Disability makes all three apprehensive. A disabled person usually requiring flexibility, assistance or structural modifications is very easily assumed to be less productive, less autonomous or burdensome even.

More than politeness is required to treat someone “like everyone else”. It is required to maintain equal expectations. The individuals with disabilities should not be infantilised, praised overly for ordinary achievements or spoken about instead of spoken to. Acceptance without condescension and assistance without assumptions should be offered. Equality of regard demands respecting autonomy even when assistance and support is required and assuming capability unless informed differently. When the language of sympathy replaces the language of rights, interactions that are supposed to appear as kind reinforce a ranking: the one who helped and the one who needed help or, the “normal” one and the exceptional one.

It is imperative to differentiate between what is being termed as “equality” and what is termed as “sameness”. A common response for discomfort is “We should treat everyone the same.” However, that sameness in a structurally unequal environment only serves to maintain the advantage. For eg., a building with only stairs is treating everyone the same, yet it remains inaccessible to some. Equality requires a different kind of support; a thoughtful design.

It is to be recognised that variations in humans is ordinary and not exceptional. Support needs to be understood as participation and not privilege.

The society already demonstrates that it is capable of separating assistance from hierarchy in other contexts like adjustable chairs for the variation in heights, translation services because of variation in languages, eyeglasses because of variation in eyesight etc. where adjustments is not dramatised or treated as charity.

Why, then, is disability not included in such normalisation?

The shift that is required to make disability inclusive is fundamentally attitudinal. Instead of perceiving disability as a defining constraint, it must be viewed as a component of human diversity. Assumption of voluntarism should be practiced instead of assumption of reliance. Resources helping to satisfy our expectations should be provided instead of decreasing them. Making someone feel welcome instead of different is what inclusivity aims for. It is about ensuring that diversity or difference does not result in respect that is lessened or diminished.

The goal of inclusivity is not to erase the differences or pretending that the disability does not exist or it does not matter. It is nor the romanticising of struggle. It is simply to demolish the hierarchy that tends to rank the differences as inferior. Accommodation should not threaten the dignity thereby triggering discomfort. A ramp for wheelchair should not be treated as a favour, or extra time in exams should not be perceived as an advantage since they are mere mechanisms that help societal institutions correct their own narrow designs. Without dismantling this hierarchy, inclusion risks becoming symbolic that is present in policy but absent in experience.

Ultimately, if individuals with disabilities need to feel included in its truest sense, they must experience equality both in perception and access. They must be directly addressed, sincerely evaluated and supported stigma-free. Their wants and needs must be met without being exaggerated into the indicators of otherness.

*Lastly, Inclusion is not about requiring extraordinary kindness. It is about requiring only **ordinary respect**.*

When support is a routine and dignity is intact, disability is no longer a part of the hierarchy. It will simply be one variation in the broad umbrella of the human experience, which is accommodated, acknowledged and treated with the same regard as any other.



*That is where inclusivity exists: not the sympathy,
but within equality.*

PERFORMATIVE INCLUSION

JOHANN SOLANKI

Reality television presents itself as an authentic depiction of everyday life. Programs featuring competition, social interaction, and unscripted events are marketed as reflections of real people navigating real situations.

However, reality television is better understood as a media institution rather than a neutral mirror of society.

Institutions develop stable norms, rules, routines, and incentives that shape how they operate. In the case of reality television, these incentives are primarily driven by audience engagement, TRP ratings, clicks, views and emotional storytelling.

Because of these incentives, reality television constructs narratives that are compelling rather than necessarily accurate. Editing, production design, and storytelling decisions determine which moments become visible to viewers. **As a result, the “reality” presented on screen is a curated version of events shaped by institutional priorities.** When disability enters this environment, many activists deem such representation as ‘performative’. The show signals diversity by including disabled contestants, but the institutional structure of the program may not meaningfully accommodate them.

Stigma and the Construction of the “Inspiring” Contestant

Sociologist Erving Goffman provides a useful lens to understand this dynamic. In his work, ‘Stigma: Notes on the Management of Spoiled Identity’, he argues that individuals with socially stigmatized identities must constantly manage how they are perceived in public settings. Disability, within many societies, is often treated as a stigmatized attribute that marks individuals as different from the social norm.

Reality television frequently manages this stigma by transforming disabled contestants into inspirational figures. Instead of normalizing disability as one aspect of human diversity, shows frame contestants through narratives of perseverance, struggle, and triumph. These stories can generate emotional engagement among viewers, but **they also reinforce the idea that disability must be overcome in order for individuals to be socially valued.**

This narrative framing shifts attention away from the structural environment of the competition itself. Instead of asking whether the format of the show creates barriers, audiences are encouraged to focus on the contestant's personal determination. In this sense,

stigma is managed not by removing structural disadvantages but by reframing them as opportunities for inspirational storytelling.

Institutional Constraints and the Limits of Inclusion

The institutional structure of reality television also shapes how disability is represented. Competitive formats, production schedules, and standardized challenges are designed to create drama and unpredictability. However, these structures are often built around assumptions of able-bodied participants.

The presence of disabled contestants does not necessarily lead to a redesign of these structures. Instead, institutions frequently maintain the existing format while incorporating diverse participants into it. This approach creates what can be best described as 'symbolic inclusion' where representation is deemed to demonstrate diversity but does not fundamentally alter the system.

A notable example occurred in *Big Brother 25*, which featured deaf contestant Matt Klotz. While his casting was widely celebrated as a step toward inclusion, some competitions relied heavily on auditory instructions or sound cues. These design features placed Matt at a disadvantage compared to other contestants who could easily hear the signals or directions involved in the challenge.

Importantly, the broadcast version of the show did not strongly highlight these accessibility barriers. Instead, the narrative emphasized Matt's resilience and ability to adapt. From an institutional perspective, this editing choice preserves the show's image as a fair competition while avoiding a deeper discussion about accessibility within the format itself.

Media Framing and the Illusion of Equal Competition

Reality television relies heavily on framing, the process through which producers and editors select certain moments to present while excluding others. This framing shapes how audiences interpret events and social dynamics within the show.

When structural barriers faced by disabled contestants are minimized or omitted, the audience sees a competition that appears equally accessible to all participants. The absence of visible barriers reinforces the idea that success or failure depends solely on individual ability. In reality, however, the structure of the competition may privilege contestants who fit the assumed norm around which the show was designed.

This pattern can be observed across multiple reality television formats. In the United States, the modeling competition America's Next Top Model featured deaf contestant Nyle DiMarco. His success on the show was widely celebrated, and his narrative was framed as a story of perseverance and inspiration. While this visibility challenged certain stereotypes about disability, the program largely focused on his individual determination rather than examining structural barriers faced by deaf individuals in the fashion industry. We must also highlight how the concept of **pretty privilege** played a crucial role in setting such narratives. Shows cast attractive and fit individuals who happened to be disabled which aligns with reality TV's surface-level diversity.

Similarly, competitive reality shows often frame disabled contestants through emotional storytelling segments that highlight personal struggles and achievements. These narratives create powerful television moments but can obscure broader social questions about accessibility, discrimination, and institutional design.

Representation Without Structural Change

The inclusion of disabled contestants in reality television demonstrates that media institutions are increasingly aware of the importance of diversity and representation. However, the structure of these programs often remains unchanged. As a result, representation may occur without meaningful transformation in how social realities are portrayed.

From a sociological perspective, this dynamic illustrates how institutions manage social expectations while preserving existing formats. Disability becomes visible on screen, but the conditions that shape participation are rarely examined.

Instead of confronting structural inequalities, reality television frequently converts them into narratives of personal resilience.

In this sense, reality television reveals an important paradox. Shows that claim to depict reality may simultaneously produce simplified and selective representations of social life. When disability is incorporated into this environment, inclusion can become performative, creating the appearance of progress while leaving deeper institutional structures intact.

THE PATIENCE WE PRESCRIBE

PRITHVIRAJ SURI

In the corporate and academic world, "inclusion" has become a bit of a buzzword. We see it everywhere: in mission statements, glossy brochures, and annual "awareness" days. It's spoken about with such moral certainty that you'd think we've already figured it out. But for those actually living with a disability, **inclusion isn't a slogan-it's an exhausting, ongoing negotiation.** And it usually comes with one unspoken rule: you have to be patient. We ask people to be patient with buildings that aren't ready, attitudes that haven't changed, and systems that are "getting there." Most of all, we ask them to be patient while everyone else "learns" how to be decent. This raises a tough question: Why is the person already dealing with exclusion the one expected to wait?

It's Not the Person; It's the Room

We use words like "special needs" or "accommodation," which sound helpful, but they actually shift the blame. When a lecture hall doesn't have a ramp, we don't say the building is broken; we say the wheelchair user "requires an accommodation." If an office doesn't use accessible software, we don't say the workplace is narrow-minded; we say the employee "needs extra support." By framing it this way, we make the person the "problem" to be solved, rather than the environment.

*True inclusion isn't about doing someone a favour; it's about redesigning a world that was built **without them in mind.***

Who Actually Feels the Discomfort?

Diversity sounds great on paper, but in reality, it creates friction. The problem is that this friction isn't shared equally. A student waiting for assistive technology might lose weeks of study time. To the university, that's a "procedural delay." To the student, that's their future hanging in the balance. It's a strange paradox: the people who have been left out the longest are the ones we ask to be the most "understanding" when things move slowly.

The Invisible Labour at Home

When institutions fail to adapt, families pick up the slack. This isn't just "family support"-it's a massive amount of unpaid, often invisible work that usually falls on women. Mothers, sisters, and daughters become full-time advocates, therapists, and negotiators. In many communities, this burden deepens gender gaps. A disabled woman might find herself overprotected or confined under the guise of "safety," making her world even smaller. We can't talk about disability without talking about gender justice.

The Trap of Being "Inspirational"

We often see disability through two lenses: it's either a "tragedy" to be pitied or an "inspiration" to be applauded. Both are exhausting. The "inspiration" narrative demands that a disabled person be a superhero just to be seen as equal. But

the most radical thing someone can ask for is the right to be ordinary.

To have a bad day, to be mediocre, or to fail without it being a "reflection" on their disability. Inclusion shouldn't require extraordinary resilience; it should just require a seat at the table.

Community: A Double-Edged Sword

We like to think "community" is the cure for institutional coldness. And it can be- neighbours who help out and friends who advocate are life-changing. But communities can also be judgmental. **In tight-knit circles, being "different" can lead to gossip, shame, or unwanted surveillance.** Real community isn't about "helping" someone from above; it's about shared accountability. As the saying goes: "Nothing about us without us."

Moving Beyond the Checklist

Inclusion isn't a "one and done" task. You don't just build a ramp and check a box. If a student gets into a top university but can't join the informal networking events where jobs are actually found, that's not inclusion-it's window dressing. True inclusion means sharing power. It means the people in charge need to be the ones who get uncomfortable for a change. It's about structural humility: admitting the system is flawed and being willing to change the rules of the game.

The Case for Impatience

We're taught that patience is a virtue, but when it's forced on you, it becomes a way to delay justice.

Maybe the answer isn't more patience, but a healthy dose of impatience. Not anger, but clarity. The kind of impatience that refuses to accept "maybe next year" as an answer. Every day spent waiting for dignity is a day of life lost.



Making Belonging Real

Inclusion shouldn't be a test of how much a person can endure. It should be a redistribution of responsibility.

Places like NMIMS, and society in general, need to move past "accommodating" people and start making them feel they truly belong. That means the burden of change has to shift to the people who built the walls, not the people trying to get through them.

Inclusion isn't a gift we give to others. It's a practice we have to live every single day

SOCIAL INCLUSION AND COMMUNITY SUPPORT

AADI KADAM

Inclusion is not an act of charity; it is an act of justice. A society reveals its moral depth not by how it celebrates strength, but by how it responds to vulnerability. Disability inclusion is often framed in terms of ramps, reservations, or rehabilitation. Yet true inclusion goes far deeper—into attitudes, power structures, gender roles, and the everyday interactions that shape dignity. **Social inclusion demands that persons with disabilities are not merely accommodated but valued as equal participants in community life.** To achieve this, we must confront stigma, interrogate social norms, recognize intersectionality, address gendered burdens and violence, and strengthen social casework, family support systems, community interventions, and media representation.

Stigma and the Weight of Social Norms

Stigma is the invisible barrier that precedes every physical one.

In many communities, disability is still associated with shame, superstition, or divine punishment. Families may hide disabled members to avoid social judgment. Children with disabilities are often excluded from schools not because of incapacity, but because of assumptions about their abilities. These attitudes **stem from rigid social norms that equate productivity with worth and independence with success.** Such norms fail to recognize that dependency is a universal human experience.

Every person relies on others at different stages of life—infancy, illness, old age. Disability simply makes this interdependence more visible. Social inclusion begins when communities redefine “normal” as diversity rather than uniformity. Education systems, workplaces, and public institutions must actively challenge stereotypes and create environments where difference is not merely tolerated but respected.

Intersectionality: When Identities Overlap

Disability does not exist in isolation. It intersects with caste, class, gender, rural–urban divides, and other social identities. A woman with a disability in a rural, low-income setting may face compounded marginalization, limited access to education, restricted mobility, and economic dependency.

Intersectionality teaches us that exclusion multiplies when identities overlap. Policies that address disability without considering these layered disadvantages risk benefiting only a privileged minority.

For example, accessible infrastructure in cities is valuable, but what of disabled persons in remote villages without transportation? Scholarships are meaningful, but what of those who drop out early due to discrimination? Inclusion must therefore be contextual and sensitive to social hierarchies. Community-level data, participatory planning, and representation of marginalized voices in decision-making are critical.

Gendered Burdens of Disability and Care

Disability is deeply gendered. Women and girls with disabilities are more vulnerable to neglect, forced sterilization, denial of reproductive rights, and exclusion from inheritance. At the same time, caregiving responsibilities disproportionately fall on women—mothers, sisters, daughters—who often sacrifice education and employment to provide unpaid care. This “**care burden**” is socially expected yet economically invisible.

Care work, though foundational to society, is undervalued. Strengthening social inclusion requires recognizing caregivers as stakeholders. Support groups, respite care services, skill training, and financial assistance can reduce burnout and restore agency. When caregivers are supported, persons with disabilities receive better care, and families become more resilient.

Violence and Hidden Vulnerabilities

Violence against persons with disabilities remains underreported and inadequately addressed. Communication barriers, social isolation, and dependency increase vulnerability to abuse. Women and children with disabilities are at particular risk of sexual violence, often by caregivers or acquaintances.

The silence surrounding such violence reflects societal discomfort in acknowledging both disability and sexuality.

Addressing this issue demands systemic change: accessible reporting mechanisms, trained police personnel, sensitized healthcare workers, and inclusive legal processes. Community vigilance committees and awareness campaigns can shift attitudes from pity to protection grounded in rights. *Justice must be accessible, not abstract.*

Strengthening Social Casework and Family Support

Professional social casework plays a transformative role in disability inclusion. Caseworkers act as bridges—connecting families to welfare schemes, educational opportunities, assistive technologies, and counselling services. Effective casework is holistic, considering the individual's emotional, social, and economic context rather than focusing solely on impairment.

Family-cantered approaches are equally important. Counselling can help families move from denial or guilt to acceptance and empowerment. Parent training programs equip families with skills to support development and independence. When families are informed and confident, they become advocates rather than passive recipients of aid.

Government schemes must be simplified and made accessible. Many families struggle with complex documentation processes. Digital platforms, mobile outreach units, and local help desks can ensure that entitlements reach those who need them most.

Community Interventions: From Isolation to Belonging

Inclusion flourishes at the community level. Schools that adopt inclusive education practices foster empathy among all children. Workplaces that provide reasonable accommodations demonstrate that productivity and diversity are compatible. Local self-help groups and cooperatives can integrate persons with disabilities into income-generating activities.

Community-based rehabilitation (CBR) models emphasize participation, local resource mobilization, and empowerment. Instead of isolating individuals in institutions, CBR strengthens community capacity to support diverse needs. Peer networks and disability-led organizations further amplify voices and promote leadership.

Religious institutions, youth clubs, and neighbourhood associations can serve as platforms for awareness and solidarity. Inclusion must be woven into everyday community life rather than confined to specialized programs.

Media Representation: Shaping Public Consciousness

Media shapes perception. When disability is portrayed only as tragedy or inspiration, it reduces complex lives to simplistic narratives. Persons with disabilities are either pitied or placed on pedestals as “superheroes.” Both extremes deny normalcy.

Responsible media representation should depict persons with disabilities as multidimensional individuals—students, professionals, parents, artists—whose identities extend beyond impairment. Inclusion in films, advertisements, newsrooms, and digital platforms can normalize diversity. Representation behind the camera is equally vital; storytellers with disabilities bring authenticity and nuance.

Social media campaigns and influencer partnerships can challenge stigma and promote positive role models. Language matters—terms that emphasize personhood rather than limitation reinforce dignity.

Toward a Culture of Belonging

Disability inclusion is not a sectoral issue; it is a societal commitment. Laws and policies lay the foundation, but attitudes build the house. A truly inclusive community recognizes that diversity strengthens rather than fragments society. It invests in support systems that uphold dignity, autonomy, and participation.

Social inclusion requires a shift from a charity-based model to a rights-based framework. Persons with disabilities are not beneficiaries of goodwill; they are citizens entitled to equality. Families are not burdens; they are partners. Communities are not passive observers; they are agents of change.

In the end, inclusion is measured not by the number of ramps constructed or schemes launched, but by whether a child with a disability can dream without fear, whether a woman with a disability can walk safely in her neighbourhood, whether a caregiver feels supported rather than isolated, and whether society sees disability not as deficiency but as diversity.

A just society does not ask whether persons with disabilities can fit into the world as it is. It asks

*how the world
must change so
that everyone
belongs.*

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